

It's Not Your Fault

A Practical Guide for Families Caring for an Aging Parent

Rob Brizzi, CDP · Certified Dementia Practitioner | Cardinal Promise Press | 8.5 × 11, full color, 29 chapters

What this book is. A calm, color-coded field guide for the adult children and spouses who are suddenly responsible for an aging parent and have no idea where to start. It walks a family from the first unsettling change, through every care setting (home health, assisted living, skilled nursing, palliative care, hospice), through the money (Medicare, Medicaid, private pay, long-term care insurance, VA), through the legal documents (powers of attorney, advance directives, DNR/POLST), and finally through the last days and the hours after a death. It is built to be opened in a crisis and read one page at a time — not cover to cover.

Why a physician should care about it. Families make their worst decisions when they are frightened and uninformed — and those decisions land back in your exam room and your hospital. This book is designed to send you a better-prepared family: one that keeps a written change log, understands the difference between observation and inpatient status, knows that a DNR is not "do not treat," and understands that hospice is a benefit to be used, not a surrender. It does not practice medicine. It teaches families to ask sharper questions and partner with their clinicians.

HOW IT WORKS

Three design choices make it usable on the hardest day of a family's life:

- 1. Color-coded urgency.** Every chapter is tagged **RED — Act Today**, **PURPLE — Watch Closely**, or **BLUE — Plan Ahead**, so a family can triage at a glance.
- 2. Wayfinding for the panicked.** A "Start Here: The First 24–72 Hours" section, five common "Find Your Situation" pathways (stroke, gradual decline, new dementia, protecting a parent's assets, choosing Medicare), and a "Choose Your Path" decision tree route a reader to the right chapter in under a minute.
- 3. The 30-second box.** Every chapter opens with a "This Chapter in 30 Seconds" box — the rule, the common myth, the red line, the exact script to use with a clinician, and what to watch for. What follows below is drawn directly from those boxes and each chapter's closing takeaway.

Why it exists. The author's father, Lou Brizzi, died on Christmas Day, 2025. Rob — who had sat at countless bedsides across a career in home health, hospice, and senior living — was not at his father's. He began journaling through the grief and the guilt, and his wife told him to turn it into a book. This is the toolkit he

wishes every family had been handed before the crisis arrived. Its title is its thesis: when a parent declines, families drown in guilt they did not earn. *It's not your fault — here is the map.*

Part 1 — Something Has Changed

The first signals, and what to do before there is a diagnosis.

CHAPTER 1 Something Has Changed

The rule: patterns deserve attention — and a sudden change should always be taken seriously. The better first question is not "Is this normal aging?" — it is: what changed, when, and is it affecting safety?

The myth: families think they need to be certain before they act. Caregiving does not require perfect certainty. It requires honest attention.

First move: write down the change with facts and dates ("got lost driving to church on March 3") — vague worry is easy for a busy doctor to dismiss; concrete observations get taken seriously.

You do not need to diagnose the problem today. The next right question is: what changed, and is anyone unsafe today?

CHAPTER 2 Falls, Weakness, and Decline

The rule: a fall is not just a fall. The better question is not whether anything broke — it is: why did the fall happen, and what needs to change so it does not happen again?

The myth: "She just tripped." Caution is not a care plan. The family's job is not to blame — it is to investigate.

Red line: a head strike on a blood thinner — even daily aspirin — means the emergency department the same day, even if they seem fine. Bleeding inside the head can show up hours or days later.

Falls, weakness, and decline are information. The next right question is: why did this happen, and what needs to change so it does not happen again?

CHAPTER 3 Memory Loss and Dementia

The rule: memory loss is not always dementia. The first question is not "Is this dementia?" It is "Is this safe?" — medication, driving, cooking, finances, and being alone.

The myth: many people with dementia can sound convincing on the phone. A parent may seem fine during a short visit but struggle when alone — the person handling the daily details often sees the truth first.

First move: write down examples with dates and ask for an evaluation. Not "Dad is getting forgetful" — "Dad got lost driving home from church, missed two medication doses this week, and paid the same bill three times."

Memory loss is about safety, judgment, and daily life — not only forgetting. The next right question is: what is no longer safe to leave to memory alone?

CHAPTER 4 Caregiver Burnout

The truth: burnout is not weakness, selfishness, or a lack of love — it is a warning sign. When a caregiver burns out, everyone becomes less safe.

The myth: "I should be able to do this." Love does not remove exhaustion. Asking for help is not failure — it is planning.

Caregiver burnout is not weakness. It is information. The next right question is: is the care plan protecting both the parent and the caregiver?

CHAPTER 5 The Family Meeting

The point: a family meeting moves the family from reacting to deciding — one place to name what changed, what is unsafe today, who is responsible for what, and what happens next.

The myth: "We can just text." Avoiding the conversation does not remove the problem. It only pushes the decision into a future crisis.

The script (the Velvet Hammer): "Thank you for sharing your thoughts. What part of the plan can you take responsibility for this week?"

A family meeting makes the facts visible, assigns real tasks, and protects both the person receiving care and the caregiver. The next right question is: who is doing what by when?

Part 2 — Care Options

Every setting a family may need, what each does and does not provide, and when to consider it.

CHAPTER 6 Home Health

What it is: medical care at home — nursing, PT, OT, speech, social work, limited aide visits — ordered by a clinician for people who meet the requirements.

What it is not: all-day supervision, housekeeping, meals, or a sitter. That is private duty care (Chapter 7).

How to ask: not "order home health" — say "Can we schedule an appointment or provider evaluation to discuss this recent change and determine whether home health is appropriate?"

Home health is medical care in the home for people who meet the requirements — not private duty care, housekeeping, or all-day supervision. The next right question is: can we schedule a provider evaluation to determine whether home health is appropriate, and what support is needed if it is not?

CHAPTER 7 Private Duty Care

What it is: non-medical help in the home, usually longer blocks of hands-on help and supervision — may include bathing, dressing, meals, light housekeeping, companionship, transportation, errands, and safety checks.

What it is not: medical care. Home health is medical, intermittent, and ordered when a person meets the requirements (Chapter 6) — a family may need both.

Private duty care is daily support, supervision, and non-medical help in the home. The next right question is: what tasks are no longer safe or realistic for the family to carry alone?

CHAPTER 8 Assisted Living

What it is: a residential care setting — not a nursing home, not memory care, not home health. It may help with meals, medications, bathing, dressing, laundry, housekeeping, safety checks, transportation, activities, and daily support.

Assisted living is a care plan, not a lobby. The next right question is: can this community safely meet the needs my parent has today — and be honest about the needs it cannot?

CHAPTER 9 Memory Care

What it is: a residential setting for people with dementia or Alzheimer's who need more structure, supervision, and safety than regular assisted living can provide.

What it is not: not a punishment, not "locking someone away," not giving up — and not measured by whether the person still recognizes family or has moments of clarity.

Memory care may be needed when dementia creates safety risks that cannot be reliably managed at home or in regular assisted living. The next right question is: is the current setting safe during the moments when memory cannot protect them?

CHAPTER 10 Skilled Nursing and Rehab

What it is: short-term medical care, nursing, and therapy after a hospital stay when a person is not ready to safely return home — a decision point, not just a place to "get stronger."

How to ask: request a care conference early and get plain language — "What does that mean at home at 2 a.m. when one person is helping?"

Rehab is a decision point, not just a place to get stronger. The next right question is: what level of care will actually be safe after rehab ends?

CHAPTER 11 Palliative Care

What it is: specialized support for serious illness — symptoms, stress, goals of care, and quality of life — that can often begin earlier and be used alongside treatment.

What it is not: not hospice, not stopping treatment, not a signal that death is close — hospice is generally for people nearing the end of life who are no longer pursuing curative treatment for the terminal condition.

How to ask: "Would palliative care be appropriate to help with comfort, quality of life, goals of care, and family support?" — and if they say it is not needed, ask what support they recommend for symptom management, caregiver stress, and planning.

Palliative care is extra support for serious illness — comfort and clarity alongside treatment, not instead of it. The next right question is: would extra support help us manage symptoms, stress, and decisions?

CHAPTER 12 Hospice

What it is: comfort-focused care for people who meet eligibility requirements and are nearing the end of life — a full team (nurse, aide, social worker, chaplain, volunteers, medical director) wherever the person lives. It is not giving up.

The trade-off: while on hospice, Medicare generally stops paying for treatment intended to cure the terminal illness — care for other health conditions continues, comfort treatment continues, and the person can leave hospice at any time and return to regular Medicare coverage.

What it is not: usually not a full-time bedside caregiver — the family, and sometimes private duty caregivers, still provide much of the hands-on daily care, so plan that coverage early.

Hospice is not the family choosing death. It is the family choosing support when the illness is already changing life. The next right question is: would comfort-focused care help us care better now?

Part 3 — Paying for Care

The money maze — and the traps that quietly cost families the most.

CHAPTER 13 Medicare Basics

The one sentence: Medicare is health insurance, not long-term care insurance — it generally does not pay for long-term custodial care (bathing, dressing, toileting, meals, supervision) when that is the only care needed.

Medicare is health insurance, not long-term care insurance. It may cover certain medical or skilled services when requirements are met, but it usually does not pay for long-term daily custodial care. The next right question is: what exact care is needed, and does Medicare cover that specific care in this situation?

CHAPTER 14 Medicaid Basics

What it is: a needs-based program, administered by states, that may help pay for long-term care — nursing facility or home and community-based services — for people who meet both financial and functional or medical requirements.

What it is not: Medicare, automatic, or the same everywhere — program names, waiting lists, and applications vary by state, so do not rely on what worked for a neighbor in another state.

Medicaid may help pay for long-term care for people who meet financial and functional requirements, but the rules vary by state and program. The next right question is: which Medicaid program might apply, what are the rules in this state, and who can help us apply correctly?

CHAPTER 15 Private Pay

What it is: the family or the person receiving care pays directly from personal funds — savings, retirement income, pensions, Social Security, investments, home sale proceeds, or family contributions.

Private pay means using personal or family funds to pay directly for care that insurance or public benefits may not cover. The next right question is: what does the current care plan cost each month, and how long can we safely sustain it?

CHAPTER 16 Long-Term Care Insurance

The starting line: you either have a policy or you do not — coverage usually has to be purchased before care is needed, so do not try to create it in the middle of a crisis. (Planning ahead for yourself? Traditional and hybrid life/LTC policies are typically bought in one's 50s or 60s — premiums rise with age and health changes can close the door — so compare options with a fee-only advisor well before care is on the horizon.)

Long-term care insurance starts with one question: does a policy already exist? If yes, find out what it covers and how to start a claim. If no, move to the next payment option.

CHAPTER 17 Prescription Assistance

The core rule: "Never disqualify yourself. Ask the question. Complete the application. Let the program tell you no." — many pharmaceutical assistance programs focus primarily on income, not the value of a person's home.

Prescription assistance starts with one practical step: make the medication list, find the expensive drugs, and ask the pharmacist and prescriber what can be done safely.

CHAPTER 18 VA Benefits

The first question: "did they serve?" — if the person is a Veteran, surviving spouse of a Veteran, or dependent of a Veteran, VA benefits may matter. Do not assume. Check.

VA benefits may help some Veterans, surviving spouses, or caregivers, but eligibility depends on the person, the program, and the situation. The next right question is: did they serve, and who can help us check what VA benefits may apply?

Part 4 — The Documents That Matter

The paperwork that decides who speaks, who decides, and what is honored.

CHAPTER 19 The Important Documents Folder

What it is: one folder — paper, digital, or both — so the right person can put a hand on the right paper in minutes, not hours.

Watch for: add a HIPAA release — without one, even a devoted daughter can be told "we can't discuss that."

The important documents folder gives the family one trusted place to find medical, legal, financial, and emergency information. The next right question is: can the right person find the right document in less than ten minutes?

CHAPTER 20 Healthcare Power of Attorney

What it does: names the person — called a healthcare agent, proxy, or surrogate; the term varies by state — who can make medical decisions if your parent cannot.

Watch for: a move to another state, a changed agent or relationship, a serious diagnosis, or an old form — do not assume an old form still does the job; ask an attorney.

A Healthcare Power of Attorney names the person who can make medical decisions if your parent cannot. The next right question is: who is legally named to speak for this person if they cannot speak for themselves?

CHAPTER 21 Living Will / Advance Directive

What it does: explains what care a person would or would not want — CPR, ventilators, feeding tubes, comfort care — while the Healthcare Power of Attorney names who speaks; a family should try to have both.

Watch for: a Living Will may express wishes about CPR, but a medical order such as a DNR, MOST, or POLST may also be needed depending on the state and setting — do not assume one form does everything.

How to ask: "Can you help us understand whether the advance directive matches the medical situation we are facing now?"

A Living Will or Advance Directive helps explain what care a person would or would not want if they could not speak for themselves. The next right question is: what did Mom or Dad say they would want if the decision became hard?

CHAPTER 22 Financial Power of Attorney

What it does: names who can manage money, bills, accounts, benefits, and property if your parent cannot — love does not automatically give that authority, and neither does being the adult child.

Watch for: confirm it is durable (durable usually means it stays effective even if the person becomes incapacitated) and whether authority begins immediately or only after incapacity is documented.

A Financial Power of Attorney names the person who can handle financial matters if your parent cannot. The next right question is: who is legally authorized to manage bills, accounts, benefits, and care costs if this person cannot?

CHAPTER 23 Medical Orders — DNR, MOST, and POLST

What it does: a living will states wishes; a signed medical order — an out-of-hospital DNR, or a portable order form such as a MOST or POLST — gives instructions that care teams, including EMS, can act on; paramedics generally cannot follow a living will sitting in a folder.

What it is not: a DNR is not "do not treat" — it addresses one thing, CPR; comfort, pain relief, dignity, and medical care all continue, and the order can be changed or voided at any time.

Watch for: this chapter's worksheet is not a DNR, MOST, or POLST and has no legal or medical effect — only an order signed by a healthcare provider on your state's official form directs medical care.

A DNR, MOST, or POLST turns a person's healthcare wishes into a medical order that care teams — including EMS — can act on, but only if it is signed, current, and findable. The next right question is: if the heart stops tonight, is there a signed order that says what to do, and can the responders find it?

CHAPTER 24 Advance Care Planning Conversations

What it is: the conversation that ties the documents together — what matters most, who should speak, what care would be wanted and not wanted — before a crisis forces it.

What it is not: not one conversation forever — what someone wanted at 65 may not be what they want at 85; ask again after a serious diagnosis, a hospitalization, or a change in health.

Watch for: a conversation is guidance, a legal document records wishes, a medical order gives instructions — depending on the state and setting, a DNR, MOST, or POLST may still be needed; do not assume one form does every job.

Advance care planning helps the family understand a person's wishes before a crisis. The next right question is: have we talked honestly about what matters most, who should speak, and what care would match this person's values?

CHAPTER 25 Medication and Doctor Lists

What it is: one current list of everything actually being taken — prescriptions, over-the-counter drugs, vitamins, supplements, drops, patches, insulin — plus allergies and recently stopped medications.

How to ask: "Can we review the medication list to make sure everything is still needed, safe, and appropriate?" Do not stop medications on your own.

A current medication and doctor list helps the family and care team make safer, faster, clearer decisions. The next right question is: can we hand the care team one accurate list today?

CHAPTER 26 Emergency Contacts and Crisis Plan

What it is: one page that answers the crisis question — who do we call first? — so the caregiver can find the right number in thirty seconds.

An emergency contact and crisis plan gives the family one clear place to find who to call and what to say when something changes. The next right question is: can the caregiver find the right number in thirty seconds?

Part 5 — Hospice and the Last Days

Comfort, presence, and what actually happens — so the family is prepared, not blindsided.

CHAPTER 27 The Last Days Plan

What it is: a plan made before the final days — what to expect, who to call first, what comfort means, where the documents are, who should be present, and what to do when death occurs.

What it is not: predicting the exact moment — no one can do that. It is preparing the family so the room can become calmer, softer, and more focused on the person receiving care.

A last days plan helps the family prepare for comfort, communication, calls, documents, visitors, and what to do when death occurs. The next right question is: what can we put in place now so the final days are less chaotic and more peaceful?

CHAPTER 28 What to Expect in the Last Days

What families may see: more sleeping, less eating and drinking, breathing changes, cool hands or feet, restlessness, visions, sometimes a rally — every person is different. Even then, hearing may remain: speak as if they can hear.

The last days may bring changes in sleep, eating, drinking, breathing, skin, awareness, and comfort needs. The next right question is: what are we seeing, who should we call, and what comfort support is needed now?

CHAPTER 29 After Death

The most important thing: in most situations, there is time. The family can pause, sit, and say goodbye — and the right first call is rarely 911.

After a death, there is usually more time than families think, and the right first call is rarely 911. The next right question is: who do we call first, and what can wait until we are ready?

A NOTE FOR CLINICAL REVIEWERS

The manuscript was written from years at the bedside, and it is careful on the points where families most often go wrong: that sudden confusion is frequently reversible (infection, dehydration, medication, hypoxia) and warrants evaluation rather than resignation; that a hospital discharge is a handoff, not a finish line; that a DNR addresses CPR only while comfort and treatment continue; that hospice eligibility, the cure-versus-comfort trade-off, and the Medicare Hospice Benefit are commonly misunderstood; and that portable orders (MOST/POLST) only work if signed, current, and findable. It is explicitly educational, not medical advice, and it repeatedly sends families back to their own clinicians for decisions about their specific situation.

If the book rings true to what you see in your own practice, an endorsement — a sentence or two in your own words, in your personal capacity — would help it reach the families who need it. Thank you for reading.

What an endorsement supports. The mission of the book is simple: to help families avoid unnecessary suffering by understanding the healthcare system *before* a crisis forces them to. Most caregivers are handed no map — no one teaches a daughter how to weigh a skilled-nursing discharge, when to ask about hospice, or how to keep a parent's wishes from being lost between one facility and the next. A physician's voice on the cover tells a frightened family that the guidance inside is safe to trust. If it would help, the author is glad to provide the full 228-page manuscript, a specific chapter in your specialty, or a short call.